WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 9

Group, Family & Couples Therapy

Yalom, Minuchin, Bowen, Gottman, and Johnson

Yalom's Factors · Structural Family · Strategic · Bowen · Gottman Method · EFT-C · Group Dynamics

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Deep Study

Group, Family & Couples Therapy — Weave Psychotherapy Vol. 9

D1: Group, Family & Couples Therapy — Deep Study

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PART I: GROUP THERAPY

1. HISTORY AND FOUNDATIONS OF GROUP THERAPY

1.1 The Pioneers

Group therapy did not emerge from a single theoretical tradition. It was born from convergent experiments across medicine, psychoanalysis, and social psychology.

Joseph Hersey Pratt (1905) is considered the father of group therapy. A Boston internist, he organized "thought control classes" for tuberculosis patients — weekly meetings where patients shared their experiences, received education about their illness, and supported each other's morale. Pratt discovered that the group itself was therapeutic: patients who attended regularly had better outcomes than those who did not, independent of any medical intervention. He called this the "class method."

Jacob Moreno (1931) coined the term "group psychotherapy" and developed **psychodrama** — a method in which patients enact their conflicts on stage rather than merely talking about them. Moreno introduced the concepts of sociometry (mapping interpersonal preferences and rejections within groups), role-playing, and the "social atom" (the nucleus of interpersonal relationships surrounding each person). He opened the first psychodrama theatre in New York in 1936.

Samuel Slavson (1934) brought psychoanalytic theory to group work, founding **activity group therapy** for children and later extending it to adults. He established the American Group Psychotherapy Association (AGPA) in 1943. Slavson's contribution was demonstrating that psychoanalytic principles — transference, resistance, free association — could operate within a group setting.

Wilfred Bion (1940s) at the Tavistock Clinic developed a radically different model. Working with soldiers during World War II, Bion proposed that every group simultaneously operates at two levels: the **work group** (task-oriented,

rational) and the **basic assumption group** (unconscious, driven by primitive anxieties). He identified three basic assumption states:

Basic Assumption	Group Behaviour	Unconscious Fantasy
Dependency	The group looks to the leader for all answers	An omnipotent figure will take care of us
Fight-Flight	The group unites against an external enemy or flees from a task	Survival depends on fighting or running
Pairing	Two members form a special relationship; the group watches with hope	A messianic event or figure will save us

EXAM PEARL

Bion's three basic assumptions — dependency, fight-flight, and pairing — are among the most frequently examined concepts in group therapy. They describe unconscious group-level dynamics that interfere with the group's stated task.

S.H. Foulkes (1940s-60s) developed **group analysis** in London. Foulkes saw the group as a matrix — a network of communication where meaning emerges from the connections between members, not from any individual alone. He coined the term "group matrix" and argued that neurosis is fundamentally a disturbance in communication. The therapist's role is a "conductor" who facilitates the flow of communication rather than interpreting individual pathology.

Irvin Yalom (1970–present) synthesized the diverse group therapy traditions into a coherent, empirically grounded framework. His textbook *The Theory and Practice of Group Psychotherapy* (now in its 6th edition with Molyn Leszcz) remains the definitive work. Yalom's central contribution is the identification of 11 therapeutic factors that operate in all effective groups.

EXAM STRATEGY

For any question on group therapy history, the safe answer includes Pratt (first groups), Moreno (psychodrama, sociometry), Slavson (psychoanalytic groups), Bion (basic assumptions), Foulkes (group analysis/matrix), and Yalom (therapeutic factors). Know the approximate decades and key contributions.

2. YALOM'S 11 THERAPEUTIC FACTORS

Yalom proposed that the curative power of group therapy derives not from any single technique but from a set of interdependent therapeutic factors that operate in every effective group. These 11 factors are the most frequently examined aspect of group therapy in psychiatry examinations.

#	Factor	Definition	Clinical Example
1	**Instillation of Hope**	Seeing others at different stages of recovery fosters optimism	A newly diagnosed patient with depression sees another member who has recovered significantly
2	**Universality**	Realising one is not alone in suffering	A man with OCD discovers others share intrusive thoughts — "I thought I was the only one"
3	**Imparting Information**	Psychoeducation and direct advice from therapist or members	Substance abuse group discusses pharmacology of alcohol withdrawal
4	**Altruism**	Members benefit from helping others; purpose through giving	A member who struggled with self-worth discovers she can offer comfort to a grieving peer
5	**Corrective Recapitulation of the Primary Family Group**	The group re-creates family dynamics, offering a chance to rework early relational patterns	A woman who was invisible in her family of origin learns to assert her needs in the group
6	**Development of Socialising Techniques**	Learning basic social skills through feedback and modelling	A socially anxious member receives gentle feedback that his silence is perceived as hostility

#	Factor	Definition	Clinical Example
7	**Imitative Behaviour**	Modelling after the therapist or other members	A member adopts the therapist's calm, validating communication style in her own life
8	**Interpersonal Learning**	The group becomes a social microcosm; members enact their relational patterns and receive corrective feedback	A man who dominates conversations is confronted by peers — for the first time, he hears how his behaviour affects others
9	**Group Cohesiveness**	The sense of belonging, acceptance, and mutual valuing within the group	Members consistently attend despite logistical difficulties because "this is the only place I can be myself"
10	**Catharsis**	Expression of strong emotions within a safe, accepting context	A member weeps openly about childhood abuse and is met with empathy rather than judgment
11	**Existential Factors**	Confronting fundamental issues of existence — death, isolation, freedom, meaninglessness	A terminally ill member's honesty about mortality transforms the group's perspective on their own complaints

EXAM PEARL

Yalom's 11 therapeutic factors should be memorised as a complete list. The most commonly tested are: universality, interpersonal learning, group cohesiveness, catharsis, and existential factors. Group cohesiveness is to group therapy what the therapeutic alliance is to individual therapy — the foundational condition.

MNEMONIC

I U I A C D I I G C E

— Instillation of hope, Universality, Imparting information, Altruism, Corrective recapitulation, Development of socialising techniques, Imitative behaviour, Interpersonal learning, Group cohesiveness, Catharsis, Existential factors. Alternatively, remember: **"HUG A CRISIS ICE"** — Hope, Universality, Group cohesiveness, Altruism, Corrective recapitulation, Recapitulation (socialising), Imitative, Socialising, Interpersonal learning, Catharsis, Existential.

CLINICAL ANCHOR

Interpersonal learning is the factor that most distinguishes group therapy from individual therapy. The group functions as a social microcosm — members inevitably reproduce their characteristic relational patterns in the group. Unlike real life, the group provides immediate, honest feedback. This "here and now" focus is the engine of change in process-oriented groups.

Relative Importance by Group Type

The 11 factors do not contribute equally in every group. Their relative importance shifts depending on the group's purpose:

Group Type	Most Active Factors
Inpatient acute	Instillation of hope, universality, imparting information, group cohesiveness
Outpatient process	Interpersonal learning, catharsis, existential factors, group cohesiveness
Psychoeducation	Imparting information, universality, instillation of hope
Support groups	Universality, altruism, group cohesiveness, catharsis
DBT skills group	Imparting information, development of socialising techniques, imitative behaviour

3. GROUP COMPOSITION AND SELECTION

3.1 Inclusion and Exclusion Criteria

Not every patient benefits from group therapy. Yalom proposed that the single most important selection principle is: **the patient must be able to participate in the group task.** For a process-oriented group, this means the capacity for interpersonal interaction, self-reflection, and emotional tolerance.

General inclusion criteria:

- Motivation and willingness to participate
- Capacity for interpersonal relating
- Some capacity for self-observation
- Problems amenable to interpersonal intervention (relationship difficulties, social isolation, low self-esteem)
- Stable enough to tolerate group affect

General exclusion criteria:

- Active psychosis with disorganised thinking

- Severe antisocial personality (exploits the group, cannot empathise)
- Active suicidality requiring individual containment
- Brain injury or cognitive impairment preventing group interaction
- Inability to tolerate a group setting (severe paranoia, extreme social phobia)
- Active substance intoxication

> EXAM PEARL
>
> The most commonly cited contraindication for outpatient process group therapy is severe antisocial personality. Such patients may dominate, exploit, or traumatise other members without capacity for empathy or self-reflection.

3.2 Group Size and Structure

- **Optimal size:** 7–8 members for process-oriented outpatient groups (Yalom). Minimum 5 to maintain viability when members are absent; maximum 10 to allow each member adequate airtime.
- **Inpatient groups:** Can be larger (8–12), with modified goals (symptom management, discharge planning, psychoeducation).
- **Open vs. closed groups:** Closed groups have a fixed membership and defined duration (e.g., 12–session CBT group). Open groups accept new members on a rolling basis as others leave (typical of long-running process groups).
- **Session frequency:** Weekly for outpatient (60–90 minutes); daily for inpatient (45–60 minutes).

3.3 Heterogeneity vs. Homogeneity

Dimension	Homogeneous Groups	Heterogeneous Groups
Composition	Similar diagnosis or problem	Mixed diagnoses, diverse backgrounds
Advantage	Rapid cohesion, universality, less dropout	Richer interpersonal learning, broader social microcosm
Disadvantage	Narrower range of interaction, may collude in avoidance	Slower to gel, higher initial dropout
Best for	Psychoeducation, support, early treatment, specific diagnoses	Long-term process, interpersonal growth
Principle	Homogeneous for support, heterogeneous for change	—

> EXAM STRATEGY
>
> If asked whether groups should be homogeneous or heterogeneous, the answer depends on the goal. For symptom relief and psychoeducation, homogeneity is preferred. For interpersonal growth and personality change, heterogeneity is preferred. Yalom's rule: "Homogeneous for support, heterogeneous for interpersonal learning."

4. STAGES OF GROUP DEVELOPMENT

All groups pass through predictable developmental stages. Two models dominate the literature.

Tuckman's Model (1965)

Stage	Characteristics	Therapist Task
Forming	Orientation, tentative engagement, dependency on leader, superficial politeness, search for structure	Provide structure, set norms, reduce anxiety
Storming	Conflict, competition for dominance, challenge to the leader, frustration, testing boundaries	Normalise conflict, model tolerance, avoid retaliating
Norming	Cohesion develops, shared norms emerge, trust deepens, mutual support	Reinforce group norms, facilitate self-disclosure
Performing	Productive work, interpersonal learning, constructive confrontation, genuine intimacy	Focus on here-and-now, deepen process, interpret patterns
Adjourning	Termination, mourning, consolidation of gains, regression	Process endings, review progress, anticipate relapse

Clinical Stage Model

Stage	Key Process	Duration
Early (Initial)	Engagement, trust-building, norm-setting, here-and-now focus established	Sessions 1–6
Transition	Conflict, resistance, dominance struggles, confrontation of the therapist	Sessions 6–12
Working	Interpersonal learning, emotional exploration, cohesive and productive	Sessions 12–end
Termination	Mourning, consolidation, transfer of gains	Final 2–4 sessions

EXAM PEARL

The storming/transition phase is where most premature dropouts occur. Members who survive the conflict phase and enter the norming/working phase show the greatest therapeutic gains. The therapist's task during storming is to normalise conflict, not suppress it.

CLINICAL ANCHOR

A common beginner's error is to rescue the group from conflict during the storming phase. Conflict, if managed well, builds cohesion. The group needs to learn that disagreement does not destroy relationships — this is a corrective emotional experience for members whose families punished conflict.

5. LEADERSHIP AND THERAPIST ROLE

5.1 Leadership Styles

Lieberman, Yalom, and Miles (1973) studied encounter groups and identified four key leadership functions:

Function	Description	Too Much	Too Little
Emotional stimulation	Challenging, confronting, modelling self-disclosure	Aggressive, overwhelming	Flat, uninspiring
Caring	Warmth, acceptance, genuineness, support	Overprotective, fosters dependency	Cold, detached
Meaning attribution	Interpreting, labelling patterns, explaining process	Intellectualised, overly cerebral	No framework for understanding
Executive function	Setting norms, managing time, stopping destructive behaviour	Rigid, authoritarian	Chaotic, unsafe

EXAM PEARL

Lieberman, Yalom & Miles (1973) found that the most effective group leaders combined moderate emotional stimulation with high caring, high meaning attribution, and moderate executive function. The least effective were "aggressive stimulators" (high confrontation, low caring) — these leaders produced the most casualties.

5.2 Co-Therapy

Co-therapy involves two therapists leading a group simultaneously. Advantages include broader observation, modelling of healthy disagreement, gender balance, mutual support, and backup when one therapist is absent. Disadvantages include higher cost, the risk of splitting (members pitting therapists against each other), and the need for co-therapists to process their relationship regularly.

> CLINICAL ANCHOR
>
> If co-therapists disagree in session, this is not a failure — it is an opportunity. Openly processing disagreement between therapists models healthy conflict resolution and demonstrates that relationships can survive difference.

5.3 Common Therapist Errors

- **Rescuing the group from silence or conflict** — silence is often productive; premature intervention interrupts processing
- **Favouritism** — unconscious alliance with particular members
- **Self-disclosure without purpose** — therapist self-disclosure should be brief, relevant, and in service of the group
- **Allowing monopolising** — one member dominates while others disengage
- **Neglecting the here-and-now** — drifting into storytelling about outside events rather than processing in-group experience
- **Failing to address absences** — unexplained absences carry meaning and must be explored

6. TYPES OF GROUPS

Type	Primary Focus	Therapeutic Factors Emphasised	Examples
Psychoeducational	Information delivery, skill acquisition	Imparting information, universality, hope	Relapse prevention, medication education, illness management
Process/Interpersonal	Here-and-now interpersonal interaction	Interpersonal learning, group cohesiveness, catharsis, existential factors	Yalom-style outpatient groups
Support	Mutual aid, coping with shared adversity	Universality, altruism, group cohesiveness	Cancer support, caregiver groups, bereavement
Skills-Based	Structured teaching of specific skills	Imparting information, socialising techniques, imitative behaviour	DBT skills group, social skills training, anger management
Psychodrama	Enactment, role-reversal, emotional release	Catharsis, corrective recapitulation, interpersonal learning	Moreno's method; scenes, soliloquy, doubling, role-reversal
Self-Help/12-Step	Peer-led recovery and accountability	Universality, altruism, hope, group cohesiveness	AA, NA, SMART Recovery, Recovery Inc.

EXAM PEARL

DBT skills groups are not process groups. They are structured psychoeducational groups that teach four modules: mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness. Individual processing of emotions and crises occurs in individual DBT therapy, not in the skills group. This split is deliberate — Linehan designed it to prevent the emotional contagion and session disruption that occurs when patients with BPD process intense emotions in a group format.

PART II: FAMILY THERAPY

7. MAJOR SCHOOLS OF FAMILY THERAPY

Family therapy emerged in the 1950s from the recognition that individual psychopathology cannot be understood in isolation — it exists within and is maintained by a relational system. The "identified patient" is the symptom-bearer, not necessarily the source of dysfunction.

Comparison of Major Schools

School	Founder(s)	Core Concept	Unit of Analysis	Primary Technique	Therapist Stance
Structural	Salvador Minuchin	Boundaries, subsystems, hierarchy	Family structure	Joining, enactment, restructuring	Active, directive, joins the system
Strategic	Jay Haley, MRI Group (Watzlawick, Weakland, Fisch)	Sequences, power, homeostasis	Repetitive interaction sequences	Directives, paradoxical interventions, reframing	Expert, directive, prescriptive
Systemic/ Milan	Selvini Palazzoli, Boscolo, Cecchin, Prata	Circular causality, hypothesis, neutrality	Belief systems and meaning	Circular questioning, positive connotation, rituals	Curious, neutral, hypothesising
Bowenian	Murray Bowen	Differentiation, triangles, multigenerational transmission	Multigenerational emotional system	Genogram, coaching, I-position	Coach, non-anxious presence
Experiential	Virginia Satir, Carl Whitaker	Emotional experience, communication stances, spontaneity	Immediate affective experience	Family sculpting, communication stances, I-messages	Warm, spontaneous, personal
Narrative	Michael White, David Epston	Stories, dominant narratives, externalisation	Meaning and language	Externalising the problem, unique outcomes, re-authoring	Decentred, curious, co-author

School	Founder(s)	Core Concept	Unit of Analysis	Primary Technique	Therapist Stance
Solution-Focused	Steve de Shazer, Insoo Kim Berg	Exceptions, solutions, strengths	What works already	Miracle question, scaling, exception finding	Collaborative, future-focused

> EXAM STRATEGY
>
> For viva questions, you must be able to name the founder and one distinguishing technique for each school. The most commonly examined are Structural (Minuchin), Strategic (Haley), and Bowenian (Bowen). Narrative and Solution-Focused appear increasingly in modern examinations.

8. KEY CONCEPTS IN FAMILY THERAPY

8.1 Systems Theory

Family therapy is grounded in **general systems theory** (von Bertalanffy, 1968). A system is more than the sum of its parts — the family's properties emerge from the interactions among its members, not from the characteristics of any individual member.

Core systemic principles:

- **Circular causality** — in a system, cause and effect are not linear. A causes B, which reinforces A. A wife's nagging may be both a cause and consequence of a husband's withdrawal. Asking "who started it?" is meaningless in circular terms.
- **Homeostasis** — systems resist change. When one member improves, another may worsen to maintain the system's equilibrium. This is why individual therapy sometimes fails — the family system sabotages the patient's gains.
- **Equifinality** — the same endpoint can be reached from different starting points. Multiple pathways lead to the same family dysfunction.

- **Feedback loops** — negative feedback maintains stability (homeostasis); positive feedback amplifies deviation (escalation or crisis).

> EXAM PEARL
>
> Homeostasis explains why a patient who improves in individual therapy may relapse when returning to their family. The family system is invested in maintaining its current equilibrium, even if that equilibrium includes pathology. The "identified patient" serves a function in the system.

8.2 Boundaries

Minuchin described three types of boundaries between family subsystems:

Boundary Type	Description	Family Pattern	Clinical Consequence
Clear	Defined but permeable; appropriate exchange of information and emotion	Healthy, flexible	Adaptive functioning
Rigid	Impermeable; members are isolated and disconnected	Disengagement	Emotional isolation, independence without support, delayed help-seeking
Diffuse	Overly permeable; no differentiation between members	Enmeshment	Over-involvement, loss of autonomy, poor individuation, psychosomatic symptoms

EXAM PEARL

Minuchin proposed that enmeshed families produce psychosomatic symptoms (anorexia nervosa, asthma, diabetes) because the child's body becomes the vehicle for expressing family conflict that cannot be spoken. The "psychosomatic family" (Minuchin, Rosman & Baker, 1978) has four characteristics: enmeshment, overprotectiveness, rigidity, and lack of conflict resolution.

8.3 Triangulation

A two-person relationship under stress recruits a third person to stabilise itself. This is **triangulation** — the fundamental unit of relational stability (Bowen). The child who is drawn into a marital conflict, the mother-in-law who sides with her son against his wife, the therapist who is pulled to take sides — all are examples of triangulation. Bowen considered triangulation the most important concept in family systems theory.

8.4 Other Key Terms

Term	Definition
Identified patient (IP)	The family member who carries the symptom; "the one who brings the family to treatment"
Genogram	A multigenerational family map (typically 3 generations) showing patterns of relationship, illness, and repetition
Subsystems	Functional subunits within the family: spousal, parental, sibling
Hierarchy	The power structure of the family; clear hierarchy is adaptive
Parentification	A child is assigned an adult caretaking role
Scapegoating	One member is blamed for the family's dysfunction
Family myth	A shared, unchallenged belief that maintains homeostasis ("We are a happy family")
Double bind	Bateson's concept: contradictory messages at different levels (verbal vs. nonverbal) from which there is no escape

EXAM PEARL

The double bind (Bateson, Jackson, Haley & Weakland, 1956) was originally proposed as a communication pattern causing schizophrenia. While this etiological claim has been abandoned, the concept remains clinically useful as a description of paradoxical, "no-win" communication patterns in dysfunctional families.

9. STRUCTURAL FAMILY THERAPY

9.1 Salvador Minuchin

Structural family therapy was developed by Salvador Minuchin at the Philadelphia Child Guidance Clinic in the 1960s-70s. Minuchin worked with disadvantaged, inner-city families and with psychosomatic families (anorexia, asthma). His model is action-oriented, present-focused, and directive.

9.2 Core Premise

Symptoms arise from dysfunctional family structures — specifically, from inappropriate boundaries, coalitions, and hierarchy. Treatment involves actively restructuring these patterns within the session.

9.3 Key Techniques

Technique	Description	Purpose
Joining	The therapist enters the family system by accommodating to its style, language, and affect	Builds alliance, gains leverage for change
Enactment	The therapist asks family members to demonstrate their typical interaction pattern in session	Makes implicit patterns visible and modifiable
Restructuring	Direct intervention to change family organisation — shifting alliances, strengthening boundaries	Changes the structure that maintains symptoms
Boundary making	The therapist creates or reinforces boundaries between subsystems	Corrects enmeshment or disengagement
Unbalancing	The therapist deliberately sides with one family member to shift the power structure	Disrupts rigid hierarchies and coalitions
Intensity	Repeating, prolonging, or amplifying a message until the family registers it	Overcomes homeostatic resistance
Complementarity	Challenging the family's linear view ("he is the problem") and reframing in systemic terms	Shifts from IP to relational pattern

CLINICAL ANCHOR

Enactment is the hallmark of structural family therapy. Rather than asking the family to describe their problems (which invites rehearsed narratives), the therapist says: "Show me. Turn to your son right now and tell him what you need." The therapist then observes the actual pattern and intervenes in real time.

> EXAM PEARL
>
> Minuchin's "lunch session" for anorexia nervosa is a classic technique — the family eats together in session while the therapist observes and intervenes in the feeding dynamics. This enactment makes the enmeshment and conflict avoidance visible.

10. STRATEGIC FAMILY THERAPY

10.1 Origins

Strategic family therapy emerged from two sources: Jay Haley's work (influenced by Milton Erickson's hypnotherapy) and the Mental Research Institute (MRI) in Palo Alto (Watzlawick, Weakland, and Fisch). The MRI group focused on communication and the "attempted solutions" that maintain problems.

10.2 Core Premise

Problems persist because of the sequences of interaction around them — specifically, the "attempted solutions" that families use to solve the problem actually maintain it. Therapy involves interrupting these self-reinforcing sequences.

10.3 Key Techniques

Technique	Description	Example
Directives	Homework tasks designed to alter the sequence	"This week, when your son refuses to eat, leave the table without commenting"
Paradoxical intervention	Prescribing the symptom or the resistance	"I want you to argue for exactly 30 minutes every evening at 7 PM"
Prescribing the symptom	Asking the patient to deliberately produce the symptom	"Try to have a panic attack right now in this room"
Reframing	Relabelling behaviour to change its meaning within the system	"Your daughter's rebelliousness is actually her attempt to bring the family together"
Restraining change	The therapist discourages change, predicting relapse	"Don't change too quickly — the family isn't ready"
Ordeal therapy (Haley)	The therapist prescribes a task more burdensome than the symptom	If insomnia persists, the patient must get up and scrub floors until dawn

EXAM PEARL

Paradoxical interventions work through two mechanisms: (1) if the patient complies (produces the symptom deliberately), they gain control over it — a previously involuntary symptom becomes voluntary; (2) if the patient rebels (refuses to produce the symptom), the symptom disappears. Either outcome is therapeutic.

CLINICAL ANCHOR

Paradoxical interventions require experience and careful case formulation. They should never be used with suicidal or self-harming patients ("I want you to cut yourself tonight" is malpractice, not strategy). They work best with oppositional, control-focused presentations.

11. BOWEN FAMILY SYSTEMS THEORY

11.1 Murray Bowen

Bowen developed his theory at the National Institute of Mental Health (NIMH) in the 1950s, initially studying families of schizophrenic patients. He later expanded his model into a comprehensive theory of the family emotional system. Bowen's theory is the most multigenerational of all family therapy approaches.

11.2 Eight Interlocking Concepts

Concept	Definition
1. Differentiation of Self	The capacity to maintain one's own thinking and emotional functioning in the face of family pressure. High differentiation = can be close without being absorbed, can be separate without being cut off. Low differentiation = emotional fusion, reactivity, driven by the approval or anxiety of others. Bowen's "Differentiation of Self Scale" ranges from 0 (complete fusion) to 100 (complete differentiation — theoretical ideal).
2. Triangles	The basic unit of the emotional system. Under stress, a two-person relationship becomes unstable and pulls in a third person. Triangles can involve people, substances, or activities (work, affairs).
3. Nuclear Family Emotional System	Four patterns through which family tension is managed: (a) marital conflict, (b) dysfunction in one spouse, (c) impairment in one or more children, (d) emotional distance.
4. Family Projection Process	Parents project their anxiety and undifferentiation onto a specific child, who becomes the most symptomatic family member. The "chosen" child is often the one who is most emotionally attuned to the mother's anxiety.
5. Multigenerational Transmission Process	Differentiation levels are transmitted across generations. Children at the lowest differentiation level in each generation select partners at a similar level — over multiple generations, differentiation progressively decreases in one branch and increases in another.

Concept	Definition
6. Emotional Cutoff	The way people manage unresolved emotional issues with parents/family by reducing or totally cutting off contact. Cutoff creates the illusion of autonomy but the emotional reactivity remains — it simply gets displaced onto new relationships.
7. Sibling Position	Based on Walter Toman's work on birth order. Oldest children, youngest children, and middle children develop predictable functional characteristics that influence marital choice and family dynamics.
8. Societal Emotional Process	Bowen's extension of family systems theory to society. When societal anxiety rises (economic crisis, war), regression to lower differentiation occurs — scapegoating, polarisation, short-term thinking.

EXAM PEARL

Differentiation of self is the master concept of Bowen's theory. It is NOT the same as autonomy or independence. A highly differentiated person can be deeply emotionally connected without losing their sense of self. The opposite of differentiation is emotional fusion — being governed by the emotional field of others.

MNEMONIC

DTFN MESS

— Differentiation, Triangles, Family projection process, Nuclear family emotional system, Multigenerational transmission, Emotional cutoff, Sibling position, Societal emotional process.

CLINICAL ANCHOR

Bowen therapy is not conducted with the whole family in the room. The therapist works primarily with the most motivated individual (often one partner) to increase their differentiation. As this person becomes less reactive, the system shifts around them. Bowen called this "coaching" — the therapist helps the individual plan strategic re-engagements with their family of origin.

Structural vs. Strategic vs. Bowen: Comparison Table

Dimension	Structural (Minuchin)	Strategic (Haley/MRI)	Bowenian
Focus	Family structure (boundaries, hierarchy)	Repetitive interaction sequences	Multigenerational emotional process
Time frame	Present	Present	Multigenerational
Unit of treatment	Whole family in session	Varies (family, couple, individual)	Often one person (coaching)
Therapist role	Active director, joins the system	Expert strategist, directive	Coach, non-anxious presence
Key technique	Enactment, restructuring	Directives, paradox, reframing	Genogram, differentiation coaching
View of symptoms	Product of dysfunctional structure	Maintained by attempted solutions	Expression of low differentiation and triangulation
Insight required?	No — action changes structure	No — behaviour change is the goal	Yes — self-knowledge enables change
Duration	Short to medium term	Brief (5–20 sessions)	Long term

PART III: COUPLES THERAPY

12. GOTTMAN METHOD COUPLES THERAPY

12.1 The Research Foundation

John Gottman's approach is unique in couples therapy: it is built entirely from longitudinal observational research conducted at the University of Washington's "Love Lab" over 40+ years, studying 700+ couples across seven separate studies. Gottman can predict divorce with 91% accuracy by observing a couple interact for as little as 15 minutes.

Core finding: Marital success depends not on conflict resolution skill but on the quality of the couple's friendship.

12.2 The Four Horsemen of the Apocalypse

The four destructive communication patterns that predict divorce:

Horseman	Definition	Behavioural Markers	Antidote
1. Criticism	Global negative statement about partner's character (differs from complaint, which targets a specific behaviour)	"You always...", "You never...", "What is wrong with you?"	**Gentle start-up / Specific complaint:** "I feel [emotion] about [specific situation] and I need [positive request]"
2. Contempt	Expression of superiority and disgust; fuelled by long-simmering negative thoughts	Sarcasm, cynicism, name-calling, eye-rolling, mockery, sneering, hostile humour	**Fondness and admiration:** Build a daily culture of appreciation; scan for what partner does right
3. Defensiveness	Counter-blame: "The problem isn't me, it's you"	Innocent victim stance, whining, cross-complaining, meeting complaint with complaint	**Accept responsibility:** Even partial — "You have a point. I could have handled that better."
4. Stonewalling	Emotional withdrawal — no eye contact, no verbal cues, no facial response	Disengagement, looking away, leaving the room, emotional shutdown	**Self-soothing:** Take a 20+ minute break when physiologically flooded (HR > 100 bpm); return when calm

EXAM PEARL

Contempt is the single best predictor of divorce. It differs from criticism in that it expresses superiority and disgust from a position of moral authority. Couples who show contempt have more infectious illness — contempt literally makes you sick.

EXAM PEARL

The Four Horsemen predict divorce at 82% accuracy alone. Adding failed repair attempts raises prediction into the 90s. The difference between stable and unstable couples is not the absence of Horsemen but the success of repair attempts.

MNEMONIC

MNEMONIC: C-C-D-S — Criticism, Contempt, Defensiveness, Stonewalling. ("Criticise, Condemn, Defend, Shut down.")

12.3 The Sound Relationship House

Gottman's architectural metaphor for a healthy marriage:

Level	Component	Description
1 (Base)	Love Maps	Detailed cognitive map of partner's inner world — worries, hopes, stresses, preferences
2	Fondness & Admiration	Fundamental sense that partner is worthy of respect and affection; antidote to contempt
3	Turning Toward	Responding to bids for connection; funding the emotional bank account
4	Positive Sentiment Override (PSO)	The resultant state of levels 1–3; benefit of the doubt in ambiguous situations
5	Manage Conflict	Solvable problems (soft start-up, repair, compromise) and perpetual problems (dialogue about dreams)
6	Make Life Dreams Come True	Honour aspirations; overcome gridlock by uncovering hidden dreams
7 (Top)	Create Shared Meaning	Rituals, roles, goals, values — a "culture of two"
WALLS	Trust & Commitment	Weight-bearing walls that protect the entire structure

EXAM PEARL

The Sound Relationship House has 7 levels with Trust and Commitment as the weight-bearing walls. The first three levels (Love Maps, Fondness/Admiration, Turning Toward) build the friendship system. The top levels (Manage Conflict, Life Dreams, Shared Meaning) manage conflict and create purpose.

12.4 Key Concepts and Data Points

The 5:1 Ratio: Stable, happy marriages maintain at least 5 positive interactions for every 1 negative during conflict. This is the single most replicated finding in marital research.

Repair Attempts: Gottman's term for any statement or action that prevents negativity from escalating. Repair attempts are the single most important predictor of marital stability. Their success depends not on eloquence but on the quality of the underlying friendship (PSO).

Perpetual vs. Solvable Problems: 69% of marital problems are perpetual (fundamental differences that will never be fully resolved). The difference between happy and unhappy couples is not whether they have perpetual problems but whether they can dialogue about them with humour and acceptance — or become gridlocked.

Flooding / Diffuse Physiological Arousal (DPA): When HR exceeds 100 bpm during conflict, the individual enters fight-or-flight. Creative problem-solving becomes physiologically impossible. The only productive response is a 20+ minute self-soothing break.

Harsh Start-Up: The outcome of a conversation can be predicted from the first 3 minutes, 96% of the time. Soft start-up (accept responsibility + state feelings + describe situation + state positive need) is critical.

Bids for Connection: Married couples turn toward bids 86% of the time; couples who later divorced averaged 33%.

> CLINICAL ANCHOR
>
> When assessing a couple, ask about their early history. Positive, vivid recall = fondness/admiration system intact, marriage salvageable. Negative rewriting or inability to remember = poor prognosis. 94% predictive accuracy (Gottman's Oral History Interview).

> CLINICAL ANCHOR
>
> The Stress-Reducing Conversation (20–30 minutes daily about external stresses, NOT the marriage) is the single most effective "turning toward" activity. Rules: genuine interest, no unsolicited advice, take partner's side, validate emotions. "When you are in pain, the world stops and I listen."

13. EMOTIONALLY FOCUSED COUPLES THERAPY (EFT)

13.1 Susan Johnson and Attachment

EFT was developed by Sue Johnson and Les Greenberg in the 1980s. It is grounded in **attachment theory** (Bowlby) — the premise that adults, like children, need a secure emotional bond with a primary attachment figure. Relationship distress occurs when this bond is threatened, triggering protest behaviours (anger, anxiety, demands) or withdrawal (shutdown, distance).

13.2 Negative Interaction Cycles

Johnson identifies predictable self-reinforcing cycles that trap couples:

Cycle	Pattern	Underlying Attachment Need
Pursue-Withdraw	One partner escalates with criticism, anger, demands; the other retreats into silence	Pursuer: "Are you there for me?" Withdrawer: "Am I good enough for you?"
Withdraw-Withdraw	Both partners disengage emotionally	Both: "It's safer not to need anyone"
Attack-Attack	Both partners escalate aggressively	Both: "I must fight for any attention I can get"

EXAM PEARL

The pursue-withdraw cycle is the most common negative interaction pattern in distressed couples. The pursuer's anger is a protest behaviour (attachment cry); the withdrawer's silence is a protective strategy (avoid further injury). Both are driven by the same underlying fear: loss of the attachment bond.

13.3 The Three Stages and Nine Steps

Stage	Steps	Focus
Stage 1: De-escalation (Steps 1–4)	1. Identify the negative interaction cycle. 2. Access the underlying attachment emotions (fear, sadness, shame beneath anger). 3. Reframe the problem in terms of the cycle and unmet attachment needs. 4. Each partner owns their position in the cycle.	Stop the cycle; both partners see the pattern as the enemy, not each other
Stage 2: Restructuring the Bond (Steps 5–7)	5. Access deeper emotions and needs (vulnerability, longing). 6. Promote acceptance of partner's experience. 7. Facilitate emotional engagement — "Hold Me Tight" conversations (bonding events).	New patterns of emotional engagement; withdrawer becomes accessible, pursuer softens
Stage 3: Consolidation (Steps 8–9)	8. Facilitate new solutions to old problems (now possible because the bond is secure). 9. Consolidate new cycles of attachment and bonding.	Integrate gains; old problems become manageable from a base of secure attachment

EXAM PEARL

The pivotal moment in EFT is the "softening" (Step 7) — when the previously critical, pursuing partner, from a place of vulnerability rather than anger, reaches for the withdrawing partner and asks for what they need. When the withdrawer responds with emotional presence, a "bonding event" occurs. This is the transformative moment of EFT.

MNEMONIC

EFT's three stages map to: **"Stop, Open, Stay"** — Stop the cycle (de-escalation), Open to vulnerability (restructuring), Stay connected (consolidation).

14. BEHAVIORAL COUPLES THERAPY

14.1 Historical Context

Behavioral couples therapy (BCT) was developed by Neil Jacobson and Andrew Christensen in the 1970s-80s, rooted in social learning theory and operant conditioning. It was the first couples therapy to be empirically tested in randomised controlled trials.

14.2 Core Premise

Relationship distress results from a low ratio of positive to negative exchanges (Jacobson & Margolin, 1979). Distressed couples show:

- Low rates of rewarding behaviour
- High rates of punishing behaviour
- Coercive interaction patterns (one partner's aversive behaviour is reinforced when the other complies to stop it)

14.3 Key Techniques

Technique	Description
Behavioural exchange (BE)	Partners are assigned to increase positive behaviours toward each other — "caring days," lists of pleasing activities, behavioural contracts
Communication training	Teaching active listening, "I-statements," editing before speaking, paraphrasing, validating
Problem-solving training	Structured negotiation: define problem specifically, brainstorm solutions, evaluate each, choose one, implement, review
Acceptance work (Integrative BCT — Christensen)	When problems cannot be changed, the focus shifts to emotional acceptance, empathic joining around the problem, unified detachment from the pattern

CLINICAL ANCHOR

Jacobson's own data showed that traditional BCT achieved 35–50% improvement rates with relapse to 18–25% at one year. This honest acknowledgment of BCT's limitations led Christensen to develop Integrative Behavioral Couple Therapy (IBCT), which adds acceptance strategies — recognising that not all relationship problems are solvable.

15. EVIDENCE BASE

15.1 Couples Therapy Outcomes

Approach	Key Evidence	Limitations
Gottman Method	91% divorce prediction (7 studies). Workshop outcomes: 75% success at 12 months (640 couples), relapse 20% (vs. 30–50% standard). Prevention workshops 3× more effective than intervention for troubled couples.	Most data from Gottman's own lab; independent replication still growing
EFT	70–73% of distressed couples recover; 86% show significant improvement (Johnson et al., 1999). Effect size d = 1.3 (large). Gains maintained at 2-year follow-up. Evidence across cultures and couple types.	Less evidence for severe personality pathology, active addiction, ongoing domestic violence
BCT / IBCT	Traditional BCT: 35–50% recovery, high relapse. IBCT: superior long-term outcomes to traditional BCT at 5-year follow-up (Christensen et al., 2010).	BCT's behavioural focus may not address deeper emotional needs
Insight-Oriented Marital Therapy	Comparable to BCT in some RCTs (Snyder, Wills & Grady-Fletcher, 1991); at 4-year follow-up, 38% of BCT couples had divorced vs. 3% of insight-oriented couples.	Single study; needs replication

Approach	Key Evidence	Limitations
Group therapy	Meta-analyses show group therapy is broadly equivalent to individual therapy for most conditions (Burlingame et al., 2003). DBT skills groups are evidence-based for BPD (Linehan et al., 2006).	Process groups are harder to manualise and study

15.2 Key Research Findings

- **Gottman's 5:1 ratio** is the most replicated finding in marital research
- **Contempt** is the single best predictor of divorce and impaired immune function in couples
- **85% of heterosexual stonewallers are men** — males have more reactive cardiovascular systems and recover more slowly from stress
- **69% of marital problems are perpetual** — fundamental differences that cannot be resolved, only managed through dialogue
- **Repair attempts** predict marital stability better than the presence or absence of conflict
- **EFT has the largest effect size** (d = 1.3) of any empirically validated couples therapy
- **67% of couples** experience a significant drop in marital satisfaction after the first baby; the 33% who do not have strong love maps (Shapiro study)

> EXAM PEARL
>
> When asked "What is the most empirically supported couples therapy?", the answer depends on the metric. EFT has the largest effect sizes in RCTs. The Gottman Method has the strongest observational/predictive research base. BCT/IBCT has the longest history of controlled trials. All three are considered evidence-based.

EXAM STRATEGY

For exam purposes, remember these numbers: 91% (Gottman's divorce prediction), 5:1 (positive-to-negative ratio), 69% (perpetual problems), 96% (harsh start-up prediction from first 3 minutes), 85% (male stonewallers), 70–73% (EFT recovery rate), 35–50% (traditional BCT recovery rate).

CLINICAL ANCHOR

Couples therapy is contraindicated when there is active, ongoing intimate partner violence with a pattern of coercive control. In such cases, individual safety planning takes priority. Gottman and EFT both screen for domestic violence before beginning couples work.

Compiled for the Weave Psychotherapy series. Primary sources: Gottman JM & Silver N (2015) The Seven Principles for Making Marriage Work; Yalom ID & Leszcz M (2020) The Theory and Practice of Group Psychotherapy, 6th edition; Minuchin S (1974) Families and Family Therapy; Bowen M (1978) Family Therapy in Clinical Practice; Haley J (1976) Problem-Solving Therapy; Satir V (1967) Conjoint Family Therapy; Johnson SM (2004) The Practice of Emotionally Focused Couple Therapy; Wolberg LR (1988) The Technique of Psychotherapy, 4th edition.

02

Clinical Quick Reference

Group, Family & Couples Therapy — Weave Psychotherapy Vol. 9

D6: Clinical Quick Reference — Group, Family & Couples Therapy

1. Group Therapy at a Glance

- **Father of group therapy:** Joseph Hersey Pratt (1905) — "thought control classes" for TB patients
- **Coined "group psychotherapy":** Jacob Moreno (1931) — also developed psychodrama, sociometry
- **Psychoanalytic groups:** Samuel Slavson (1934) — founded AGPA (1943)
- **Basic assumptions:** Wilfred Bion — dependency, fight-flight, pairing (unconscious group dynamics)
- **Group analysis/matrix:** S.H. Foulkes — neurosis as communication disturbance; therapist as "conductor"
- **11 therapeutic factors:** Irvin Yalom — definitive modern framework
- **Optimal group size:** 7–8 (process group); 5 minimum, 10 maximum
- **Session format:** Outpatient: weekly, 60–90 min. Inpatient: daily, 45–60 min
- **Top contraindication:** Severe antisocial personality (exploits group, cannot empathise)
- **Open vs. closed:** Closed = fixed membership + set duration. Open = rolling admission

> EXAM PEARL
>
> Bion's three basic assumptions (dependency, fight-flight, pairing) describe unconscious group processes that interfere with the group's rational task. They represent the "work group" vs. "basic assumption group" split.

2. Yalom's 11 Therapeutic Factors — Master Table

#	Factor	Core Idea	Most Active In
1	Instillation of Hope	Seeing others recover	Inpatient, early groups
2	Universality	"I am not alone"	All groups, especially support
3	Imparting Information	Psychoeducation, advice	Psychoeducation, DBT skills
4	Altruism	Helping others heals the helper	Support, 12–step
5	Corrective Recapitulation of the Primary Family Group	Rework early family dynamics in the group	Long-term process
6	Development of Socialising Techniques	Learn social skills via feedback	Skills-based, process
7	Imitative Behaviour	Model after therapist/members	Early groups, skills
8	Interpersonal Learning	Group as social microcosm + corrective feedback	Process groups (most distinctive factor)
9	Group Cohesiveness	Belonging and acceptance (= alliance in individual therapy)	All groups (foundational)
10	Catharsis	Safe emotional expression	Process, psychodrama
11	Existential Factors	Death, isolation, freedom, meaning	Advanced process groups

EXAM PEARL

Group cohesiveness is to group therapy what the therapeutic alliance is to individual therapy — the foundational condition without which no other factor operates effectively.

3. Group Stages Comparison Table

Tuckman (1965)	Clinical Model	Key Process	Therapist Task	Dropout Risk
Forming	Early/Initial	Orientation, dependency, politeness	Provide structure, set norms	Low
Storming	Transition	Conflict, dominance struggles, challenge to leader	Normalise conflict, model tolerance	**Highest**
Norming	Working (early)	Cohesion, trust, shared norms	Reinforce norms, facilitate disclosure	Low
Performing	Working (mature)	Interpersonal learning, genuine intimacy	Here-and-now focus, interpret patterns	Minimal
Adjourning	Termination	Mourning, consolidation, regression	Process endings, review gains	N/A

EXAM PEARL

Most premature dropouts occur during the storming/transition phase. Members who survive this phase and enter the working phase show the greatest therapeutic gains. Never rescue a group from productive conflict.

4. Family Therapy Schools — Comparison Table

School	Founder(s)	Core Concept	Key Technique	Therapist Stance	Duration
Structural	Minuchin	Boundaries, hierarchy, subsystems	Joining, enactment, restructuring	Active director	Short-medium
Strategic	Haley, MRI group	Attempted solutions maintain problems	Directives, paradox, reframing	Expert strategist	Brief (5–20 sessions)
Systemic/ Milan	Selvini Palazzoli et al.	Circular causality, neutrality	Circular questioning, positive connotation	Curious, neutral	Medium
Bowenian	Murray Bowen	Differentiation of self, triangles	Genogram, coaching, I-position	Coach	Long-term
Experiential	Satir, Whitaker	Emotional experience, communication	Family sculpting, communication stances	Warm, spontaneous	Variable
Narrative	White & Epston	Dominant narratives, externalisation	Externalising, unique outcomes, re-authoring	Decentred, curious	Medium
Solution-Focused	de Shazer, Berg	Exceptions, strengths, solutions	Miracle question, scaling, exception finding	Collaborative	Brief

EXAM STRATEGY

For viva: name the founder + one distinguishing technique for each school. Most commonly examined: Structural, Strategic, Bowenian.

5. Key Family Therapy Concepts — Glossary

Concept	Definition
Systems theory	Family = more than sum of parts; properties emerge from interactions, not individuals
Circular causality	Cause and effect are reciprocal, not linear (A causes B, B reinforces A)
Homeostasis	System resists change; improvement in IP may trigger deterioration in another member
Identified patient (IP)	The symptom-bearer; the member who brings the family to treatment
Boundaries	Clear (healthy), rigid (disengaged), or diffuse (enmeshed)
Enmeshment	Over-involvement, loss of autonomy, diffuse boundaries (Minuchin)
Disengagement	Emotional isolation, rigid boundaries, delayed help-seeking
Triangulation	Two-person stress recruits a third to stabilise; Bowen's fundamental unit
Genogram	Multigenerational family map (3+ generations); patterns of illness, relationships, repetition
Parentification	Child forced into adult caretaking role
Scapegoating	One member blamed for family's dysfunction
Double bind	Bateson: contradictory messages at different levels from which there is no escape
Differentiation of self	Bowen: capacity to maintain self while emotionally connected to others

Concept	Definition
Emotional cutoff	Bowen: managing unresolved issues by cutting off contact (illusory autonomy)
Family projection process	Bowen: parents transmit anxiety onto a specific child
Multigenerational transmission	Bowen: differentiation levels transmitted across generations
Psychosomatic family	Minuchin: enmeshment + overprotectiveness + rigidity + poor conflict resolution

EXAM PEARL

Homeostasis explains why individual therapy gains collapse when a patient returns to their family. The system is invested in its current equilibrium — even pathological equilibrium.

6. Gottman's Four Horsemen + Antidotes

Horseman	What It Looks Like	Antidote
1. Criticism	"You always forget. You just don't care." (Global character attack)	**Gentle start-up:** "I feel [emotion] about [situation]. I need [request]."
2. Contempt	Sarcasm, eye-rolling, mockery, name-calling (superiority + disgust)	**Fondness & admiration:** Daily appreciation; scan for positives
3. Defensiveness	"It's not my fault — what about YOU?" (Counter-blame, victim stance)	**Accept responsibility:** "You have a point. I could have done better."
4. Stonewalling	No eye contact, no response, emotional shutdown (85% are men)	**Self-soothing:** 20+ min break when HR > 100 bpm; return when calm

Key numbers:

- Contempt = single best predictor of divorce
- Four Horsemen alone = 82% prediction accuracy
- Four Horsemen + failed repairs = 90%+ accuracy
- 5:1 ratio of positive to negative interactions in stable marriages
- 96% of conversations end on the same note they begin (start-up matters)
- C-C-D-S mnemonic: Criticise, Condemn, Defend, Shut down

> EXAM PEARL
>
> Contempt differs from criticism in that it conveys superiority and moral disgust. Criticism attacks behaviour; contempt attacks the person's worth. Contempt also predicts infectious illness in the partner receiving it.

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7. Sound Relationship House — Levels

Level	Component	What It Means
7 (Top)	Create Shared Meaning	Rituals, roles, goals, values — "culture of two"
6	Make Life Dreams Come True	Honour aspirations; overcome gridlock via hidden dreams
5	Manage Conflict	Solvable (soft start-up, repair, compromise) + Perpetual (dialogue, not resolution)
4	Positive Sentiment Override	Benefit of the doubt; result of strong friendship (levels 1–3)
3	Turning Toward	Responding to bids for connection; emotional bank account
2	Fondness & Admiration	Respect, appreciation; antidote to contempt
1 (Base)	Love Maps	Cognitive map of partner's inner world
WALLS	Trust & Commitment	Weight-bearing walls of the entire structure

Key data:

- 69% of problems are perpetual; 31% solvable
- Couples who turned toward bids: 86% (married) vs. 33% (divorced)
- 67% drop in satisfaction after first baby; protected by strong love maps
- Magic Six Hours/week maintains the relationship
- Oral History Interview predicts outcome with 94% accuracy

> CLINICAL ANCHOR
>
> The first three levels (Love Maps, Fondness/Admiration, Turning Toward) build the friendship system. Without friendship, conflict management fails — because repair attempts depend on Positive Sentiment Override, which depends on friendship.

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8. EFT Stages — Quick Card

Model: Emotionally Focused Couples Therapy (Sue Johnson, 1980s). Attachment-based. Adults need a secure bond; distress = attachment insecurity.

Most common negative cycle: Pursue-Withdraw

- Pursuer: anger = protest behaviour ("Are you there for me?")
- Withdrawer: silence = protective shutdown ("Am I good enough?")

Three Stages, Nine Steps

Stage	Steps	Goal	Key Moment
1. De-escalation	1. Identify negative cycle 2. Access underlying emotions (fear, shame beneath anger) 3. Reframe: cycle is the enemy, not each other 4. Each partner owns their position	Stop the cycle	Both partners see the pattern
2. Restructuring the Bond	5. Access deeper needs (vulnerability, longing) 6. Promote acceptance of partner's experience 7. Facilitate emotional engagement — "Hold Me Tight" conversations	Create new bonding events	**Softening** — pursuer reaches from vulnerability; withdrawer responds with presence
3. Consolidation	8. New solutions to old problems (now possible from secure base) 9. Consolidate new cycles of attachment	Integrate gains	Secure bond established

> EXAM PEARL
>
> The "softening" at Step 7 is the pivotal, transformative moment of EFT. The previously critical partner reaches from vulnerability (not anger), and the withdrawing partner responds with emotional accessibility. This is the bonding event.

Mnemonic: EFT stages = **"Stop, Open, Stay"** — Stop the cycle, Open to vulnerability, Stay connected.

9. Evidence Snapshot

Approach	Recovery Rate	Effect Size	Key Stat	Limitation
Gottman Method	75% at 12 months (workshop data)	—	91% divorce prediction accuracy; 20% relapse	Most data from Gottman's own lab
EFT	70–73% recovery	d = 1.3 (large)	86% show significant improvement; gains held at 2 years	Less evidence for severe PD, active addiction, DV
Traditional BCT	35–50%	d = 0.6–0.9	High relapse (18–25% retained at 1 year)	Behavioural focus may miss emotional needs
IBCT	Superior to BCT long-term	—	Better 5-year outcomes than BCT (Christensen, 2010)	Still limited long-term data
Group therapy	Equivalent to individual therapy (meta-analyses)	Comparable	DBT skills groups = evidence-based for BPD	Process groups harder to manualise

Contraindication for couples therapy: Active intimate partner violence with coercive control pattern. Individual safety planning takes priority.

> EXAM PEARL
>
> EFT has the largest effect size (d = 1.3) of any empirically validated couples therapy. Gottman has the strongest observational/predictive base. BCT/IBCT has the longest RCT history.

10. Viva Questions with Model Answers

Q1: What are Yalom's therapeutic factors? Which is most important?

Answer: Yalom identified 11 therapeutic factors: instillation of hope, universality, imparting information, altruism, corrective recapitulation of the primary family group, development of socialising techniques, imitative behaviour, interpersonal learning, group cohesiveness, catharsis, and existential factors. Group cohesiveness is the most foundational — it is the group equivalent of the therapeutic alliance. Without cohesiveness, no other factor can operate. In process-oriented outpatient groups, interpersonal learning is the most distinctive and powerful change mechanism.

Q2: What are Bion's basic assumptions?

Answer: Bion proposed that all groups operate simultaneously at two levels: the work group (task-oriented, rational) and the basic assumption group (unconscious, primitive). Three basic assumption states exist: (1) Dependency — the group looks to the leader as an omnipotent figure; (2) Fight-flight — the group unites against a perceived enemy or avoids the task; (3) Pairing — two members form a special dyad while the group watches with messianic hope. These assumptions interfere with the group's stated work and must be identified and interpreted.

Q3: What is the difference between enmeshment and disengagement?

Answer: Both are descriptions of boundary pathology in Minuchin's structural family therapy. Enmeshment involves diffuse boundaries where family members are over-involved in each other's lives, with poor individual autonomy. Disengagement involves rigid boundaries where members are emotionally isolated and disconnected. Enmeshed families are associated with psychosomatic symptoms; disengaged families present with delayed help-seeking and emotional isolation. Healthy families have clear boundaries — permeable enough for connection, defined enough for autonomy.

Q4: Describe the Four Horsemen and their antidotes.

Answer: Gottman identified four destructive communication patterns predicting divorce: (1) Criticism (character attack; antidote: gentle start-up with specific complaint); (2) Contempt (superiority and disgust; antidote: fondness and admiration); (3) Defensiveness (counter-blame; antidote: accepting responsibility); (4) Stonewalling (emotional shutdown; antidote: self-soothing with a 20+ minute break). Contempt is the single best predictor of divorce. The Four Horsemen alone predict divorce at 82%; adding failed repair attempts pushes accuracy above 90%.

Q5: What is the Sound Relationship House?

Answer: Gottman's architectural metaphor for a healthy marriage. Seven levels from base to top: Love Maps, Fondness and Admiration, Turning Toward, Positive Sentiment Override, Manage Conflict, Make Life Dreams Come True, and Create Shared Meaning. Trust and Commitment are the weight-bearing walls. The first three levels build the friendship system, which generates PSO — the benefit of the doubt that allows repair attempts to succeed during conflict.

Q6: Explain the pursue-withdraw cycle in EFT.

Answer: The pursue-withdraw cycle is the most common negative interaction pattern in distressed couples according to EFT (Sue Johnson). One partner escalates with criticism, anger, and demands (the pursuer), while the other retreats into silence and emotional distance (the withdrawer). Both behaviours are attachment-driven: the pursuer's anger is a protest behaviour masking the question "Are you there for me?", while the withdrawer's silence is a protective strategy against the fear of inadequacy. EFT works by accessing the vulnerable emotions beneath both positions and facilitating new patterns of emotional engagement.

Q7: How does Bowen's differentiation of self differ from autonomy?

Answer: Differentiation is not independence or emotional distance. A highly differentiated person can be deeply emotionally connected to others without losing their sense of self — they can be intimate without fusion and separate without cutoff. A person with low differentiation is either fused (governed by others' emotional states, reactive, approval-seeking) or cut off (distanced from family but still reactive, displacing unresolved emotional issues onto new relationships). Autonomy implies separation; differentiation implies the capacity to maintain self while remaining connected.

Q8: Compare structural, strategic, and Bowenian family therapy.

Answer: Structural (Minuchin) focuses on present family structure — boundaries, hierarchy, subsystems — using active techniques like joining, enactment, and restructuring. Strategic (Haley/MRI) targets repetitive interaction sequences, using directives and paradoxical interventions to interrupt self-reinforcing problem patterns. Bowenian therapy is multigenerational, focusing on differentiation of self and triangles, working primarily with one motivated individual through coaching. Structural is action-oriented in session; strategic is prescriptive via homework; Bowenian is insight-oriented and long-term.

Q9: What is the evidence for EFT?

Answer: EFT has the largest effect size (d = 1.3) of any empirically validated couples therapy. Approximately 70–73% of distressed couples recover, and 86% show significant improvement (Johnson et al., 1999). Gains are maintained at 2–year follow-up. EFT has been validated across diverse cultures and couple types. Limitations include less evidence for couples where one partner has severe personality pathology, active substance dependence, or where there is ongoing domestic violence.

Q10: When is couples therapy contraindicated?

Answer: Couples therapy is contraindicated when there is active intimate partner violence with a pattern of coercive control. In this context, conjoint therapy can be weaponised by the abusive partner, who may retaliate against disclosures made in session. Individual safety planning takes priority. Both Gottman and EFT protocols screen for domestic violence before initiating couples work. Other relative contraindications include active psychosis in one partner, active severe substance dependence, or when one partner has already firmly decided to leave the relationship and is using therapy as a vehicle for announcement.

Compiled for the Weave Psychotherapy series. Sources: Gottman & Silver (2015), Yalom & Leszcz (2020), Minuchin (1974), Bowen (1978), Haley (1976), Satir (1967), Johnson (2004), Wolberg (1988).

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